[redacted]

[redacted]

DISCHARGE SUMMARY

UHID	: 90634496	**IP NO**	: VZ1IP26/66216
Name	: [redacted]	**Bed No**	: ISO-04
Age/Gender	: 52 Years/Male	**Admission Date**	: 09/02/2026 09:07 PM
Primary Consultant	: [redacted]	**Date Of Discharge**	: 13/02/2026 09:00 AM
Discharge Type	:	**Patient Mobile**	: [redacted]

DISCHARGE TYPE:DOCTOR ADVICE

CONSULTANT: [redacted]	[redacted]
CONSULTANT: [redacted]	**SPECIALITY: MBBS, DNB, (SURGERY) FMAS, FIAGES, GENERAL LAPAROSCOPIC AND ROBOTIC SURGEON**

Diagnosis : FUC OF LIVER ABSCESS DRIANAGE,CHRONIC CALCULOUS CHOLECYSTITIS (CONTRACTED GALLBLADDER)

BRIEF HISTORY AND EXAMINATION FINDINGS ON ADMISSION:

[redacted]

ADMITTED WITH C/O PAIN IN ABDOMEN
H/O FUC OF LIVER ABSCESS
NO H/O DYSPEPSIA PRESENT
NO H/O VOMITINGS PRESENT
NO +JAUNDICE
NO H/O FEVER
O/E
: VITALS:STABLE
P/A-SOFT, MILD TENDERNESS AT RIGHT HYPOCHONDRIUM REGIONPRESENT, PIGTAIL CATHETER INSITU
.

COURSE IN HOSPITAL (INCLUDING ALL REFERRAL CONSULTANT, CLINICAL EXAMINATION AND OPINIONS): ADMITTED WITH ABOVE COMPLAINTS AND DIAGNOSED AS CASE OF FUC OF LIVER ABSCESS DRIANAGE,CHRONIC CALCULOUS CHOLECYSTITIS (CONTRACTED GALLBLADDER)
ROBOTIC CHOLECYSTECTOMY WAS DONE UNDER GA ON 10-2-2026
POST OPERATIVE PERIOD UNEVENTFUL. HIS GENERAL CONDITIONIMPROVED GRADUALLY,PASSED STOOL AND TOLERATING DIET. NOW PATIENT IS BEINGDISCHARGED IN STABLE CONDITION WITH FOLLOWING ADVICE.

SURGERIES/PROCEDURE PERFORMED (Chronologically):NA
SURGERY DATE: 10/02/2026
NAME OF THE SURGERY /PROCEDURE: ROBOTIC CHOLECYSTECTOMY

[redacted]

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DETAILS OF SURGERY/PROCEDURE AND FINDINGS:
OPERATIVE FINDINGS: SMALLBOWEL DENSLY ADHERENT TO ANTERIOR ABDOMINAL WALL PIGTAILCATHETERNOTED IN LEFT LOBE OF LIVER AND DENSLY ADHERENT TO OMETUM GALLBLADDER CONTRACTED AND OCCUPIED BY LARGESTONE AND DENSE LY ADHERENT WITH OMENTUMAND DUODENUM PRESENT CALOTS TRIANGLE- FROZENAND DENSE ADHESIONS C/S-GB WALL THICKNED LARGE STONE WITH MUCUS PRESENT OPERATIVESTEPS- -PNEUMOPERITONEUMACHEIVED THROUGH VERRES NEEDLE AT PALMAR'S POINT -PORTSPLACED - ROBOTICARMS DOCKING DONE -FINDINGSNOTED -ADHESIONS SEPARATED INVIEW OF DENSE ADHESIONS AND FROZEN CALOTS TRIANGLE GALLBLADDER OPENED STONE REMOVED AND STUMPCLOSED WITH 3-0 PDS -GALLBLADDER MOBILIZED FROM LIVER BED - HAEMOSTASISSECURED -24CHEST DRAIN PLACED IN SUBHEPATIC REGION -CLOSED WITH 2-0 NYLON

ANAESTHESIA DETAILS:
TYPE OF ANAESTHESIA: GA
NAME OF ANAESTHETIST: [redacted]
CRITICAL EVENTS IN OT IN DETAILS:

SIGNIFICANT MEDICATION GIVEN HOSPITALIZATION

DOSE/ROUTE/FREQUENCY/DURATION

Details of Implants: Type/Brand/Size/Unique number (sticker(s) may be used):

Significant Laboratory investigations done during admission chronologically :Enclose all reports

Test	Value	Unit	Reference Range
Date - 11/02/2026			
CBP WITH PLATELET COUNT			
HAEMOGLOBIN	9.8	g/dL	(13 - 17)
PCV	31.1	%	(40 - 54)
RBC COUNT	3.58	millions/ul	(4.5 - 5.5)
MCV	86.8	fL	(83 - 101)
MCH	27.4	pg	(27 - 32)
MCHC	31.6	g/dL	(31.5 - 34.5)
RDW-CV	16.8	%	(11.6 - 14)
TWBC	13820	number/ul	(4000 - 10000)

[redacted]

Prepared By: [redacted]

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DIFFERENTIAL LEUCOCYTE COUNT			
NEUTROPHILS	74.22	%	(40 - 80)
LYMPHOCYTES	19.08	%	(25 - 45)
MONOCYTES	5.30	%	(2 - 9)
EOSINOPHILS	0.85	%	(1 - 6)
BASOPHILS	0.55	%	(<1 - 2)
PLATELET COUNT	368100	/Cumm	(150000 - 410000)
L F T (TOTAL)			
TOTAL BILIRUBIN	1.0	mg/dL	(0.3 - 1.2)
DIRECT BILIRUBIN	0.2	mg/dL	(0 - 0.2)
SERUM ALKALINE PHOSPHATASE	164	U/L	(Male : 43 115 Female : 33 - 98)
S G P T	36	U/L	(Male : <50 Female : <35)
S G O T	57	U/L	(Male : <40 Female : <35)
SERUM TOTAL PROTEINS	8.2	gm/dl	(6.6 - 8.3)
SERUM ALBUMIN	3.0		(-)
GLOBULIN	5.2	gm/dl	(-)
A G RATIO	0.5	.	(-)

Significant Radiology investigations done during admission chronologically:Enclosed all reports

Date - 09/02/2026

[redacted]

Prepared By: [redacted]

UHID	: 90634496	**IP NO**	: VZ1IP26/66216
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Pacs Result

X-RAY CHEST PA VIEW

FINDINGS:

Heart size is normal.

Both lungs are clear.

Both the hila are normal.

Both domes of diaphragm and CP angles are clear.

Soft tissue chest wall and bone cage - Normal.

Date - 12/02/2026

Pacs Result

ULTRASOUND ABDOMEN

Liver is normal in size (13.5 cm) with increased echotexture. No focal lesion seen. Intrahepatic biliary and vascular radicles are normal. Common bile duct is normal in caliber. Portal vein is normal.

Minimal perihepatic fluid noted.

No evidence of splenomegaly (12.1 cm), lymphadenopathy is seen.

Prepared By: [redacted]

[redacted]

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Gallbladder is not visualized - Post cholecystectomy status.

Hypoechoic collection measuring 3.5 x 2.3 cm noted in gallbladder fossa ? likely post operative changes.

Pancreatic size, outline and echogenicity are normal. Peripancreatic planes are well defined. No calcification or ductal dilatation is seen.

Right kidney measures 11.5 x 4.5 cm. Cortical echogenicity is normal. Corticomedullary differentiation is normal. Pelvicalyceal system is normal.

Left kidney measures 12.1 x 4.9 cm. Cortical echogenicity is normal. Corticomedullary differentiation is normal. Pelvicalyceal system is normal.

Urinary bladder is well distended and normal.
Prostate measures 4.7 x 3.1 x 2.9 cm (volume 22 cc).
IMPRESSION:
* **Grade I fatty liver.**
* **Minimal perihepatic fluid.**
* **Hypoechoic collection in gallbladder fossa ? likely post operative changes.**

REPORTED BY: [redacted]

Condition at discharge in detail:

[redacted]

Prepared By: [redacted]

UHID	: 90634496	**IP NO**	: VZ1IP26/66216
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STABLE

Discharge Diet plan:
AVOID SPICY AND NON VEG DIET

Discharge physiotherapy plan:

Discharge medication including inputs of all referral doctors, continuation of earlier regular medication,
No abbreviations
Prescription in uppercase with name of drug, form, dose, frequency, duration, route, relation to food

Name of the Drug	Dose	Route	Frequency	Duration	Relationship to Meal
1.TAB.JIOCEF	500	ORAL	BD	7 DAYS	AFTER FOOD
2. CAP.GEORAB DSR		ORAL	BD	15 DAYS	BEFORE FOOD
3.TAB.GEONAC-P		ORAL	BD	5 DAYS	AFTER FOOD
4. CAP MECOGOLD	1CAP	ORAL	OD	15 DAYS	AFTER FOOD 2PM
5. T.BACT OINTMENT					EXTERNAL APPELICATION
6. PEPTAMEN PROTEIN POWDER	2SCOOPS	ORAL	EVERY 6TH HRLY		AFTER FOOD

Special discharge needs or any other details:

Follow up plan with date, any investigations and with whom:
Review after 2 weeks AT SURGICAL GASTRO OPD FOR SUTURE REMOVAL
Not to lift heavy weights for 6 weeks

ICD classification:

Discharge instructions on when and how to obtain urgent care:
PAIN ABDOMEN, VOMITINGS

In case of any bleed or pain in Surgery site please contact [redacted] **Emergency Department** [redacted]
For Doctor appointment [redacted] **Ambulance service AVAILABLE.**

DOCTOR WHO PREP [redacted]	**PRIMARY CONSULTANT WHO APPROVED DISCHARGE SUMMARY**
Name of Doctor:	**Name of Doctor:**
Signature:	**Signature:**

Prepared By: [redacted]

UHID	: 90634496	IP NO	: VZ1IP26/66216
Name	: [redacted]	Bed No	: ISO-04

Medical Reg No:	Medical Reg No:

[redacted]

PATIENT / SURROGATE: -

I have been explained and understood, regarding the discharge summary in detail, including medication and any special needs in my local language and have received copies of discharge summary, laboratory and radiology reports.

NAME	
RELATIONSHIP	
SIGNATURE	
CONTACT DETAILS	